

New Generation English B· Diagnostic Test

C1 (Advanced Academic) B· Listening

LISTENING

Lecture: the diagnostic temptation in social commentary

Script:

In today's lecture I want to focus on what I will call the diagnostic temptation in social commentary. The case I will use is the recent discussion of loneliness as an epidemic.

The vocabulary of epidemic is unusually powerful. It moves a problem from the realm of personal misfortune into the realm of public health, which means budgets, agencies and policy. That is often useful. The 2023 Surgeon General advisory in the United States, for instance, would have attracted very little attention if it had used softer language. By choosing the word epidemic, it secured a level of seriousness that purely sociological framings rarely achieve.

However, the same vocabulary can mislead. An epidemic implies a pathogen, a vector and individual cases. Loneliness, when examined carefully, behaves more like an ecological condition than a contagion. It emerges from the way cities are built, from how working hours have shifted, from which institutions have been allowed to weaken. It is not transmitted; it is produced.

This distinction matters because the two framings recommend very different interventions. A pathological framing favours individualised responses: therapy, medication, perhaps an app. An ecological framing favours structural responses: walkable neighbourhoods, public libraries, time-protective labour laws. Both can coexist, but they are not equivalent.

The deeper question — and this is where I want you to push beyond conventional readings — is who benefits from each framing. Clinical and technological interventions are easier to monetise and easier to study. Structural interventions are slower, less visible and politically uncomfortable, because they imply that current city design, current labour culture, and current public spending priorities have been quietly producing the very condition they now claim to treat.

I am not suggesting that individual treatment is useless. I am suggesting that, taken in isolation, it acts as a kind of moral comfort. It allows societies to be seen to be doing something about loneliness without changing the structures that produce it. That, I would argue, is precisely the conservatism of a diagnostic vocabulary applied to a structural problem.

1. What does the lecturer call the diagnostic temptation?

A) A tendency to treat structural problems as medical conditions.

B) A reluctance to use scientific terms.

C) An unwillingness to consult patients.

2. Why does the lecturer say the vocabulary of epidemic is powerful?

A) It moves a problem from personal misfortune into public health, with budgets and agencies.

B) It sounds friendly.

C) It is easy for journalists to misunderstand.

